WOMEN'S HEALTHCARE ASSOCIATES NEWBERG

Rural Health Clinic

POLICY AND PROCEDURE MANUAL

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Introduction:

This policy manual is specific to the operations of Women's Healthcare Associates Newberg, a Provider Based Rural Health Clinic (PBRHC) owned and operated by Women's Healthcare Associates.

Women's Healthcare Associates- Newberg
1003 Providence Drive, Suite 340 &310
Newberg, OR 97132
(503) 538-2698

Operational Leadership Protocol

The Practice Manager is responsible for overseeing the day-to-day operations of the clinic. In the absence of the Practice Manager, responsibility for managing daily operations will be delegated as follows:

1. **First Alternate**: Patient Services Supervisor
2. **Second Alternate**: Patient Care Supervisor

Each alternate will assume operational leadership duties until the Practice Manager returns, or a temporary replacement is appointed.

The Medical Director is responsible for Medical Direction and quality of care for the RHC.

A sustained and substantial effort is made to keep the RHC in full compliance with applicable State and Federal laws and this policy manual is one of the tools that helps ensure compliance of the RHC.

This manual will be reviewed on an annual basis by an Annual Evaluation Committee consisting of the following members: Practice Manager, Advanced Practice Clinician, Site Clinician Lead, and a representative from the Nurse Manager team.

- Chief Medical Officer: Marni Carlyle, MD
- Practice Manager: Michele Perez
- Site Clinician Lead: Elisa Barske, MD
- Advanced Practice Clinician (APC): Heather McCook
- Nurse Manager: Amy Palmer, RN

Policy & Procedure Manual Review History:

FORMULATED September 29, 2025
REVIEWED
REVISED

Mission Statement:
We are committed to remaining clinician owned and led so we can deliver exceptional obstetrics, gynecology, midwifery, and wellness care grounded in relationships, clinical quality, responsiveness to our community, and care for our people.

Our Values:

SERVICE

- We put people at the center of care, striving to create a lasting positive impact through every interaction.
- We seek to understand and respond to the unique needs of the communities we serve and support.
- We are committed to serving all people, including the most vulnerable, with compassion and dignity.

DIVERSITY

- We believe a diverse workforce and inclusive culture make us stronger and improve the care we provide.
- We seek out and value different perspectives, experiences, voices, and contributions.
- We strive to eliminate disparities in opportunity, access and outcomes through education, reflection, and action.

INTEGRITY

- We build trust through honest, transparent, and consistent communication.
- We seek and act on feedback to continuously improve.
- We hold ourselves accountable, aligning our actions with our values.

EXCELLENCE

- We pursue continuous learning and professional growth.
- We bring together diverse perspectives and expertise to achieve the highest standards.
- We challenge ourselves and each other to do our best work every day.

TEAMWORK

- We recognize each person's role as essential to achieving our shared mission.
- We build trust and resilience through collaboration and connection.
- We support one another through challenges and celebrate collective success.

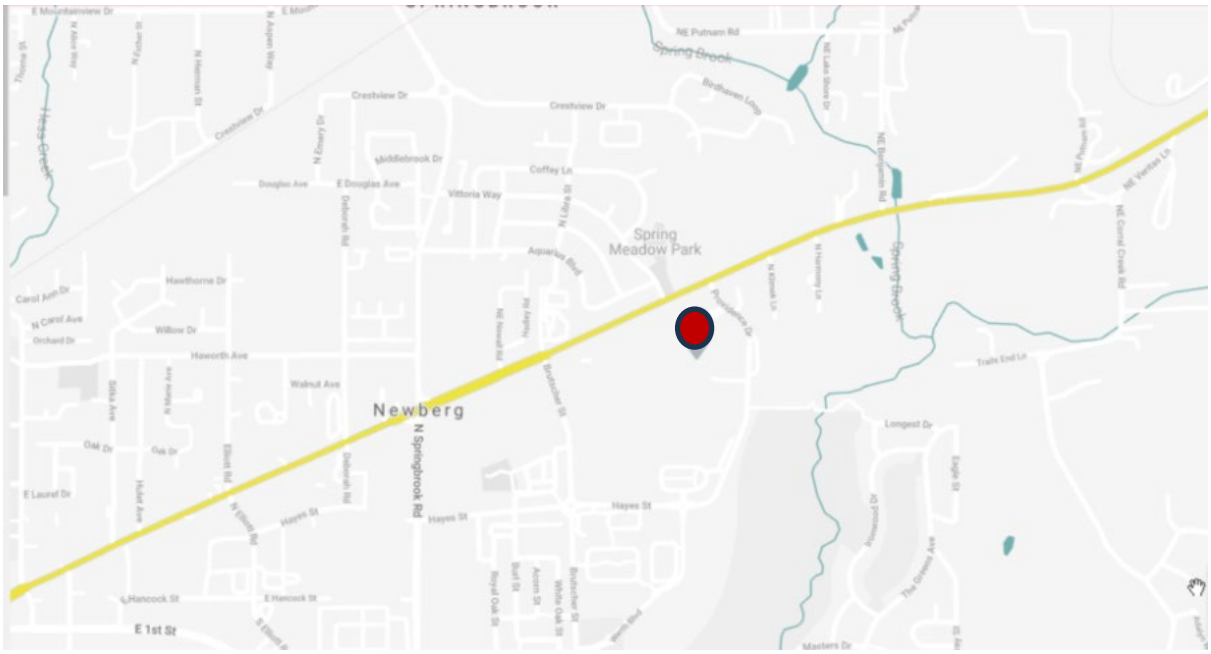
BELONGING

- We cultivate an environment where everyone feels seen, valued, and empowered to contribute.
- We support each person's path toward growth, fulfillment, and purpose.
- We work to ensure everyone—staff and patients alike—feels respected, safe, and connected.

WELLBEING

- We recognize that caring for patients begins with caring for our teams and one another.
- We promote ways of working that support long-term health, purpose, and sustainability.
- We create moments of joy, fun, connection, and levity—because they matter.

Clinic Location:
WHA Newberg is located at 1003 Providence Drive, Suite 340, Newberg, OR 97132: in Yamhill County, Oregon.
Telephone: (503) 538-2698
Fax: 503-416-2269
Website: www.whallc.com



WHA Newberg **Office Hours:**

Monday	8:00 – 5:00
Tuesday	8:00 – 5:00
Wednesday	8:00 – 5:00
Thursday	8:00 – 5:00
Friday	8:00 – 5:00
Saturday	Closed
Sunday	Closed

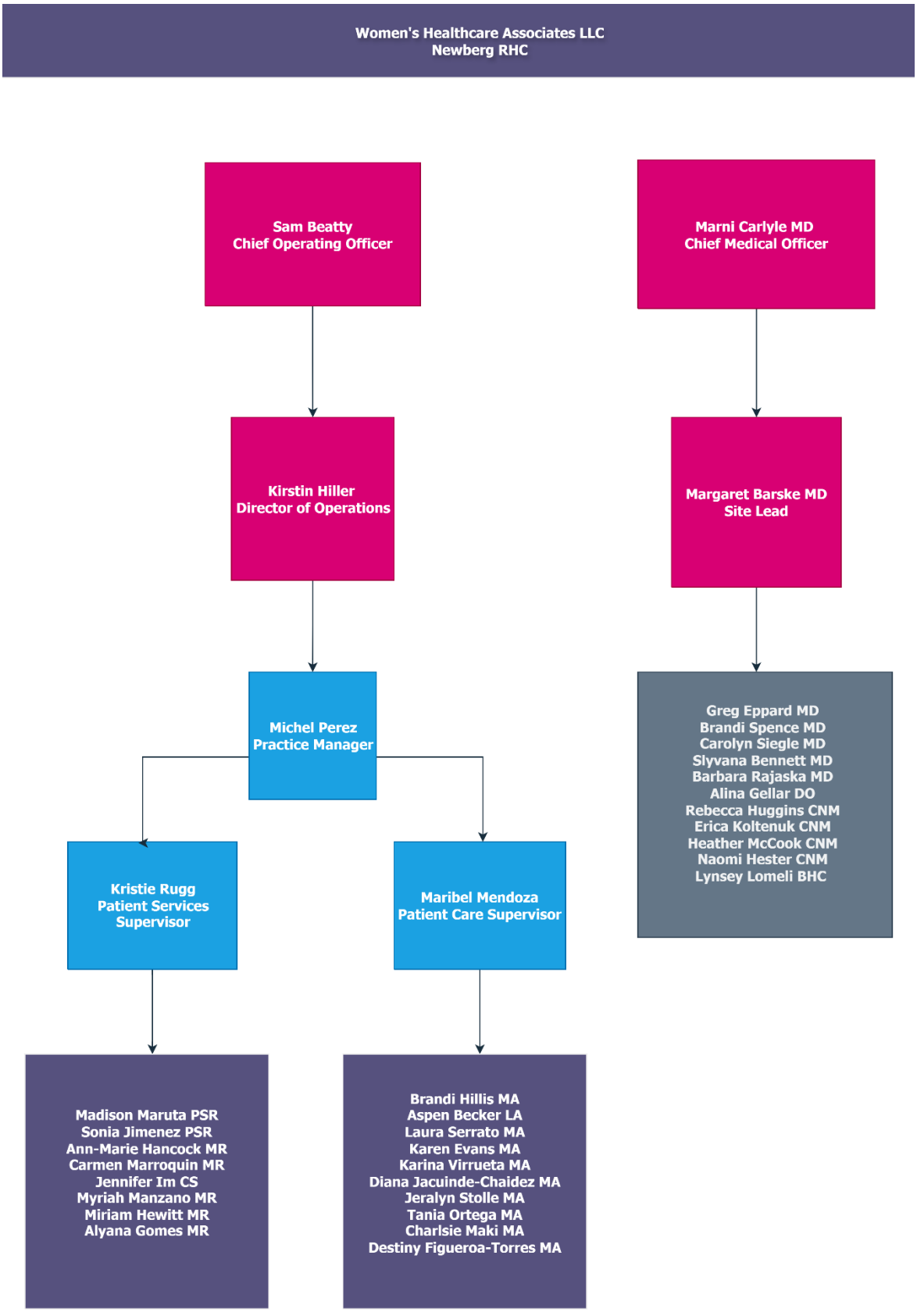
After Hours:
In the event of a medical emergency after regular clinic hours, patients seeking either urgent or emergent medical care will be instructed via WHA Newberg's recorded phone message to dial 911. Emergency care is available at Providence Newberg Hospital 365 days per year 24 hours per day.

Clinic Holidays / Closures:

Notifications of clinic closures will be posted one week prior to the closure. WHA Newberg is closed if any of the following dates / holidays fall on a day that WHA Newberg would routinely be opened on:

Christmas Eve - December 24th	Close at 12:00pm
Christmas – December 25th	Full Day
New Year's Day – January 1st	Full Day
Martin Luther King Day	Full Day
Fourth of July – July 4th	Full Day
Memorial Day	Full Day
Labor Day	Full Day
Thanksgiving	Full Day
Day after Thanksgiving	Full Day

Woman's Healthcare Associate Newberg RHC Organizational Chart



WOMEN'S HEALTHCARE ASSOCIATES, LLC

TITLE: **Patient Care Policy & Procedures** **POLICY RHC 0-1**

APPLIES TO: Newberg RHC Staff and Clinicians

POLICY: The following policies were developed by the medical providers of Women's Healthcare Associates (WHA) Newberg. It is the policy of the WHA Newberg to provide the best and most appropriate and medically necessary services to the members of Yamhill County.

It is the policy of the WHA Newberg Rural Health Clinic (RHC) to ensure that all patient care policies are developed and reviewed in accordance with the regulatory standards set forth under 42 CFR §491.9 for Rural Health Clinics. The clinic shall maintain a structured process for the initial development and biennial review of all patient care policies to ensure quality, safety, and compliance with federal regulations.

PROCEDURE: **Patient Care Services**
The RHC will provide the following direct services at the clinic site, making use of the services of both the physicians and advanced practice clinicians (refer to job descriptions).

Professional Services
- Office Visits
- Ultrasound Services
- Physical Exams
- Preventive Exam and Services
- Patient Education and Counseling
- Blood Pressure Checks
- Immunizations / Injections

Clinical Procedures
- Catheterization (bladder)
- Cauterization
- Excision of Small and Large Skin Lesions
- Incision and Drainage (simple and uncomplicated)
- IUD insertions/removals
- EMB
- Nexplanon insertion/removals
- Colposcopy
- LEEP

Injections / Immunizations / Procedures offered at WHA Newberg are:
- Influenza Immunization
- Tuberculosis Screening
- Tdap Immunization
- Prescribed Injectable Medications (i.e. Depo-Provera)
- RSV Immunization
- HPV Immunization

Laboratory Services
- Provide laboratory services appropriate to the medical needs of the patient, using the facilities located onsite or at Hospital.

- Basic laboratory procedures will be performed at WHA Newberg that are applicable under the clinics CLIA Waived Laboratory Certificate (PPM).
- Services will be performed by appropriately trained clinic personnel.

Laboratory Services available on site:

- Urinalysis, by dipstick
- Routine diagnostic testing
- Specialized lab services
- Blood sugar
- Pregnancy testing (urine)
- Primary culturing for transmittal to reference lab

Laboratory Services collected on-site with off-site analysis:

- Pap Smear
- Punch Biopsy
- Endometrial Biopsy
- Urine Culture
- Culture & Sensitivity

Laboratory tests that are not available at WHA Newberg will be collected at the Providence Newberg Lab.

Guidelines for Medical Management of Health Care Problems

- All records entered into the WHA Newberg electronic medical record are permanent. All healthcare records shall be well documented, containing sufficient data to correctly assess and respond to medical problems which are reviewed. See Policy O-19 Records Retention 2012.03.02.docx
- All consultations and referrals will be made by the physician or advanced practice clinician after consultations with the patient. Such consultation / referral will be documented in the patient record.
- Clinician arranges for or refers patients to needed services that cannot be provided at clinic
- Referrals shall be coordinated by WHA Newberg's Patient Services personnel.

Reference Resources

To ensure consistent, high-quality care in the rural health clinic setting, the clinic will identify and document the medical reference resources agreed upon by the Medical Director, Site lead and APC. These resources serve as the foundation for clinical decision-making and may include current medical textbooks, written protocols, clinical guidelines, and reputable electronic tools.

Approved reference resources may include, but are not limited to:

- ACOG (American College of Obstetricians and Gynecologists) Practice Guidelines
- ACNM (American College of Nurse-Midwives) Clinical Guidelines
- MFM (Maternal-Fetal Medicine) Guidelines for high-risk obstetric care
- UpToDate, or other peer-reviewed electronic medical references
- Clinic-specific policies and protocols developed in collaboration with the Medical Director

These resources are reviewed regularly and updated as needed to reflect current standards of care. All clinical staff are expected to use these references when providing patient care and documenting clinical decisions.

Off-Site Services

It is the policy of WHA Newberg to provide the following services to the members of Yamhill County off site:

- Inpatient and Outpatient Hospital Services and Procedures
- Obstetrical Procedures Including Deliveries and C-Sections
- Emergency Room Coverage

Policy Provisions

Development of Patient Care Policies:

- Patient care policies shall be developed by an advisory group composed of, at a minimum:
    - A physician;
    - A physician assistant (PA) or nurse practitioner (NP**)**; and
    - At least one individual who is not a member of the clinic staff, to provide an external perspective.
- The advisory group shall ensure that the policies are consistent with accepted clinical standards and reflect the specific needs of the clinic's patient population.
- This requirement is in accordance with 42 CFR §491.9(b)(2).

Biennial Review of Patient Care Policies

- All patient care policies shall be reviewed at least biennially (every two years) by the advisory group to ensure continued relevance, accuracy, and compliance with current clinical guidelines and regulations.
- The review process must be documented and include any updates or revisions made to the policies as a result of the review.
- This requirement is in accordance with 42 CFR §491.9(b)(4).

Responsibilities

- Clinic Administrator**:** Responsible for ensuring compliance with this policy and scheduling regular reviews.
- Advisory Group: Responsible for the development, review, and recommendation of updates to patient care policies.
- Medical Director**:** Provides final clinical oversight and approval of patient care policies.

WOMEN'S HEALTHCARE ASSOCIATES, LLC

TITLE: **Presence of a Medical Provider** **POLICY RHC 0-2**

APPLIES TO: Newberg RHC Staff and Clinicians

POLICY: A medical provider shall be present on the premises at all times during which the Rural Health Clinic (RHC) is open to the public

PROCEDURE: It is the policy of WHA Newberg to always have a medical provider on the clinic premises that the RHC is open to the public.

The hours of operation of the RHC are as follows:

Monday: 8:00am to 5:00pm
Tuesday: 8:00am to 5:00pm
Wednesday: 8:00am to 5:00pm
Thursday: 8:00am to 5:00pm
Friday: 8:00am to 5:00pm
Saturday: CLOSED

Should patient arrive prior to the provider arriving on premises, the patient shall be allowed to wait in the front lobby. Business office services may be provided without a provider on the premises, but no patient care shall be performed until the provider arrives. Patient care services include, but are not limited to injections, Blood Pressure Checks, Rooming a Patient, etc. In the event of an emergency, clinical staff will follow the C-13 Medical Emergency Response 2024.03.05.docx

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WOMEN'S HEALTHCARE ASSOCIATES, LLC

TITLE: **Personnel Qualifications, Training, and Compliance Documentation**
POLICY RHC 0-3

APPLIES TO: Newberg RHC Staff and Clinicians

POLICY: The clinic employs and maintains qualified personnel and provides appropriate orientation, training, and continuing education in accordance with federal and state regulations and consistent with the scope of services offered to beneficiaries. Documentation of qualifications, training and compliance is maintained in the employees' personnel files.

PROCEDURE: **Hiring and Credential Verification**

- Human Resources and/or Credentialing verify all required education, licensure, certification, and prior work experience before hire.
- Verification of credentials is documented in the employee's personnel file.
- Background checks, reference checks, and pre-employment health screenings (e.g., Hepatitis B immunity status, and Tuberculosis screening) are completed.

Orientation

- All new employees participate in an orientation program appropriate to their role, which includes **s**afety and emergency procedures and full on-site safety walkthrough by a member of the safety committee.

Annual Compliance Training

- Emergency Codes
- Infection Prevention and Control (Specific to Clinical Staff)
- Bloodborne Pathogen Standards
- Tuberculosis Control
- Hazard Communication

Training and Competency Validation

- Each department provides job-specific orientation, training, and support, including mentorship or preceptorship, for both staff and clinicians.
- **Competency assessments** are completed for clinical staff at least **annually** per regulatory requirements and will be re-evaluated **more frequently if there are practice changes or performance concerns.**

Continuing Education

- The clinic supports and requires participation in ongoing education and training to maintain clinical skills, licensure, and certification.
- Human Resources and Credentialing track completion of required continuing education (e.g., BLS) and identify any deficiencies.
- Newberg Rural Health Clinic has a continuing education reimbursement program for our staff: HR-47 Qualified Education Reimbursement Program QERP 2024.02.15.docx

Review and Updates

- Human Resources policies and this procedure are reviewed at least biennially by clinic leadership to ensure compliance.
- Any necessary revisions are approved by clinic leadership.

Responsibilities

- **Human Resources / Credentialing/Training Team**
  - Verify credentials, licensure, certification, references, and prior work experience (resumes/CV).
  - Maintain documentation of verification, orientation, training, and continuing education.
  - Track compliance with ongoing credentialing and continuing education requirements.
- **Department Managers / Supervisors**
  - Ensure orientation, job-specific training, preceptorship or mentorship is assigned.
  - Competency validation is completed before staff practice independently.
  - Monitor staff performance and identify training needs.
  - Document and address deficiencies or retraining requirements.
- **Safety Committee Members**
  - Lead site-specific Safety Walkthroughs for new employees.
  - Provide updates on safety and emergency protocols.
- **Employees**
  - Maintain current licensure and certifications as required for their role.
  - Participate fully in orientation, training, and competency validation.
  - Complete assigned continuing education and compliance training on time.
- **Clinic Leadership**
  - Review human resources policies and this procedure at least biennially.
  - Approve updates or revisions as needed to maintain compliance with regulatory requirements.

WOMEN'S HEALTHCARE ASSOCIATES, LLC

TITLE: Rural Health Clinic (RHC) Corporate Compliance Plan **POLICY RHC 0-4**

APPLIES TO: Newberg RHC Staff and Clinicians

POLICY: All staff at WHA's RHC shall abide by all laws and ethical conduct standards as stipulated by federal and state law as well as appropriate accrediting bodies.

PROCEDURE:

Mission
We are committed to remaining clinician owned and led so we can deliver exceptional so we can deliver exceptional obstetrics, gynecology, midwifery, and wellness care grounded in relationships, clinical quality, responsiveness to our community, and care for our people.

Vision
Every person has the care and knowledge they need to live a full and healthy life.

Code of Conduct – WHA Policy HR-2 (HR-2 Code of Conduct 2025.03.03.docx) describes WHA's Code of Conduct and outlines the expected ethical standards at WHA.

WHA prohibits employment and contracting with individuals or companies which have been convicted of a felony offense related to healthcare. WHA will not retaliate against employees or clinicians who make a good faith report of a perceived violation of WHA's Code of Conduct.

Leadership, Oversight and Reporting
WHA's Chief Executive Officer is designated as the RHC Compliance Officer. Employees and clinicians are asked to report any perceived violations of the Code of Conduct or any concerns regarding unethical behavior or practices to their manager or any other manager.

The CEO, Director of HR, Risk and Compliance Manager, CFO, Accounting Manager and Senior Revenue Cycle Manager may receive concerns or complaints related to compliance or ethical behavior from any manager or from employees and clinicians directly. If there is a concern regarding the CEO's behavior or practices related to this policy, it may be directed to the Management Committee President. Employees and clinicians may also utilize WHA's compliance reporting hotline, ComplyLine, via the link on Sharepoint to make anonymous reports, and these will be investigated.

Policies and Procedures – WHA's Policy O-27 (Coding Compliance Plan), IS01-05 (HIPAA / Information Security Policies), with HR-2 (Code of Conduct) and HR-6 (Confidentiality) describe expected conduct at WHA.

Training and Education
All employees and clinicians will receive training on this plan and will receive annual Fraud, Waste and Abuse Training.

Risk Assessment, Quality Improvement and Auditing
Regular auditing will occur by the HR, IT, Patient Financial Services, Operations and Risk and Compliance Teams based on areas identified as high priority. WHA undergoes an annual financial audit by a CPA firm. When necessary, investigations are undertaken to evaluate concerns and mitigation or corrective action is applied as appropriate.

A risk assessment has been completed to identify the RHC's areas of vulnerability in regards to the Compliance Plan.

Enforcement and Corrective Action
When Non-Compliance is identified, corrective action will be applied, up to and including termination and notification of regulatory bodies or law enforcement, if indicated.

WOMEN'S HEALTHCARE ASSOCIATES, LLC

TITLE: **Preventative Maintenance and Repair Policy** **POLICY RHC 0-5**

APPLIES TO: Newberg RHC Staff and Clinicians

POLICY: It is the policy of WHA and the Newberg RHC to maintain all medical, surgical, and clinical equipment in safe and effective working order through a structured Preventative Maintenance and Repair Program.

PURPOSE: To establish a standardized process for:

- Scheduled inspection, testing, and maintenance of equipment
- Prompt identification, reporting, and repair of malfunctioning or damaged instruments and equipment
- Maintaining complete documentation to support compliance and lifecycle planning

Responsibilities

- **Clinical Staff:** Immediately reports equipment malfunction or damage and remove unsafe equipment from service and tag appropriately.
- **Purchasing / Facilities / Patient Care Supervisors (PCS) / Practice Managers (PM): C**ollaborate to coordinate vendor maintenance, contracts, repairs, replacements, and loaner equipment
- **Clinic Managers:** Maintain oversight of documentation and vendor contracts. Provides support for delays or logistics issues.
- **Purchasing:** Maintain repair logs, shipping records, and vendor communication
- **NB RHC PM:** Reviews compliance with the preventative maintenance program biennially.

PROCEDURE: **Inventory and Scheduling**

- **Purchasing/Facilities:** Maintain master inventory of all equipment requiring preventative maintenance in the digital repository.
- **Contracted Biomedical Team:** Assigns preventative maintenance intervals (e.g., monthly, quarterly, annually) based on manufacturer recommendations, regulatory standards, and clinic use.
- **Purchasing/Facilites:** Coordinates with our Biomedical team and Practice Managers for upcoming inspections.

Preventative Maintenance

- **Contracted Biomedical Team:** Coordinates with Practice Mangers to conduct inspections, cleaning, calibration, and testing per manufacturer guidelines.
- **Contracted Biomedical Team:** Document all maintenance, including date, technician/vendor, work performed, and equipment condition.
- For specialty Equipment refer to policy C-8 Maintenance and Electrical Safety Testing of Ultrasound Equipment 2024.03.13.docx

Equipment Repair Process

When damage or malfunction is discovered:

- Report it immediately
- If discovered during a procedure, remove from service and replace if possible.
- Clearly tag equipment as "Do Not Use or Repair."
- Log the issue in the electronic Repair Log and notify the manager.
- Coordinate repairs or replacements through Purchasing and Practice Managers using approved vendors

Repair Request Logging and Responsibilities

- **Repair L**og:
  - Staff or PCS complete "Identifying the Need" and "Logging the Repair" sections.
  - Purchasing completes "Send for Repair" and "Tracking" sections.
  - PCS completes "Receiving and Testing" upon return.
- **Roles:**
  - PCS: In-house troubleshooting (e.g., power supply, recalibration)
  - Purchasing/PCS/PM: Vendor communication, repair/replacement, loaner coordination
  - Site leadership: Coordinate patient rescheduling if needed due to equipment unavailability

Sending for Repair

- If the equipment or instruments cannot be decontaminated, proper labeling and notification is required.
  - OSHA and CDC guidelines require instruments to be decontaminated (cleaned and **sterilized**) before transport.
- Document shipping details: tracking number, vendor, estimated return date
- Retain a copy of the service request and vendor correspondence
- Notify affected departments of status and any available loaners
- Purchasing/PCS/PM follow up with vendor as needed
- Review repair costs with Purchasing and PM

Documentation of Preventative Maintenance

- Documentation of Preventative Maintenance
- Maintain electronic maintenance logs for each piece of equipment, including:
  - Equipment name, make/model, serial number
  - Preventative maintenance schedule
  - Service records and repair history
- Tag equipment with the maintenance date and next scheduled inspection date
- Retain all records per the O-19 Records Retention 2012.03.02.docx policy
- PCS inspects and tests returned items before returning them to service
- Update the Repair Log with return date, repair details, and final status (fixed, replaced, still faulty)
- Notify staff when the item is cleared and available for use

Review and Oversite

- The preventative maintenance program is reviewed biennially by the Clinic Manager and the onsite QI Committee.
- Updates are made to the inventory, vendor contracts, and schedules as needed.

WOMEN'S HEALTHCARE ASSOCIATES, LLC

TITLE: **Storage, Handling, and Prescription of Drugs, Biologicals, and Supplies**
POLICY RHC 0-5

APPLIES TO: Newberg RHC Staff and Clinicians

POLICY: The clinic maintains written procedures to ensure that all drugs, biologicals, and supplies are stored, handled, and in a manner that maintains safety, effectiveness, and compliance with legal requirements.

PURPOSE: To establish standardized procedures for the safe storage, handling, and disposal of drugs, biologicals, and supplies in compliance with Federal and State regulations, including 42 CFR §491.9(b)(3)(iii), DEA requirements, and the Poison Prevention Packaging Act.

PROCEDURE: **Medication Storage & Use Policy**
To ensure safe and compliant handling of all medications, the clinic follows these standards:

- **Original Containers**
  - All drugs are stored in their original manufacturer's containers.
  - Vials may be removed from outer cartons, but the manufacturer's label must remain intact to preserve labeling and product integrity.
- **Multi-Dose Vials**
  - Multi-dose vials must be stored and used in accordance with manufacturer's instructions.
  - If a multi-dose vial is taken into a patient's exam room, it must be discarded after use to prevent contamination.
- **Single-Dose Vials**
  - Single-dose vials are never reused.
  - Any remaining medication must be discarded immediately after the vial is entered.

Monitoring & Documentation Procedures

- **Daily Checks:**
  - Staff verify and document storage temperatures twice daily, even with the clinics continuous monitoring in place.

Logs

- Temperature records and alarm response documentation are maintained in a secure log (paper and electronic) for a minimum of 3 years.
- Logs are reviewed monthly by the Patient Care Supervisor to ensure compliance.

Calibration & Maintenance:

- Continuous monitoring devices are calibrated and tested per manufacturer recommendations, with records kept on file. Refer to the Preventative Maintenance and Repair Policy.docx

Inventory Management

- Inventory of all drugs, biologicals, and supplies is reviewed monthly for:
  - Manufacturer expiration dates.
  - Beyond-use dating.

Outdated, deteriorated, or adulterated drugs/biologicals

- They are removed immediately from active stock.
- Are stored separately in a designated, clearly marked area.
- Are disposed of in compliance with applicable State and Federal laws.

Environmental Controls

- Drugs and biologicals are stored under conditions that maintain appropriate humidity, temperature, and light.
- Out-of-range temperature alerts are automatically sent to PCS, Practice Managers, and Nurse Managers to ensure prompt action and protection of medication integrity. Vaccine Management.docx

Storage standards:

- No pharmaceuticals are stored in the door of refrigerators/freezers.
- Medications stored in the refrigerators are clearly labeled
- Dormitory/bar-style refrigerator/freezers are prohibited for medication/vaccine storage.

Recordkeeping:

- Receipt logs and reconcilable distribution logs are maintained in Epic.

Prescriptions

- Nurse Practitioners (NPs) and Physician Assistants (PAs) practicing at the clinic may prescribe medications in accordance with their professional licensure, DEA registration, and applicable state and federal regulations.
- NPs, licensed by the Oregon State Board of Nursing, have independent prescriptive authority and may prescribe medications without physician supervision or co-signature. PAs, licensed by the Oregon Medical Board, may prescribe medications as authorized in their board-approved Practice Agreement with a supervising physician; co-signature of individual prescriptions is not required.
- All prescriptions must be documented in the patient's medical record in accordance with clinic policies and applicable laws. Refer to: Medication Prescribing Policy.docx

Dispensing to Patients

- Medications are not directly dispensed by NB Rural Health Clinicians.

References

• 42 CFR §491.9(b)(3)(iii)
• DEA Practitioner's Manual
• Poison Prevention Packaging Act of 1970
• Oregon State Board of Pharmacy Regulations

WOMEN'S HEALTHCARE ASSOCIATES, LLC

TITLE: **Medication Prescribing** **POLICY RHC 0-6**

APPLIES TO: This policy applies to all WHA Employees, Members, Associates,Hospitalists, including all Newberg Rural Health Clinicians and staff

PURPOSE: To establish standards for prescribing medications at Women's Healthcare Associates (WHA) that comply with Federal, State of Oregon, and Rural Health Clinic (RHC) requirements. This policy ensures safe, appropriate, and legally compliant prescribing practices across all WHA clinic locations.

POLICY: **Providing Prescriptions**

- **Authorization to prescribe**
  - Licensed physicians, nurse practitioners (NPs), certified nurse midwives (CNMs) and physician assistants (PAs) credentialed at WHA may prescribe medications within their defined scope of practice and in compliance with state licensure requirements.
  - Controlled substances may only be prescribed by providers with valid DEA registration.
- **Prescription Standards**
  - Prescriptions must include patient name, date of birth, date, medication name, dosage, quantity, instructions, provider signature, and DEA number if applicable.
  - Electronic prescribing (E-prescribing) is the standard method; If the E-prescribing system (Epic, the state PDMP, or the pharmacy's system) is unavailable, prescribers may issue a paper prescription, so patient care isn't delayed.
  - Prescriptive authority is exercised under the supervision of the WHA Medical Director, consistent with RHC requirements.
  - Refills may be authorized in accordance with state law, DEA schedules, and clinician judgment.
  - Registered Nurses (RNs) may review refill requests prior to processing. If care is current meaning the patient has been evaluated by or had a treatment plan discussion with the prescribing clinician within the past year-and the plan of care is clear to continue, the RN may process the refill in alignment with the documented plan of care and per Oregon RN scope of practice. Any questions, changes in condition, or unclear plans must be referred to the prescribing clinician.
- **Controlled Substances**
  - Controlled substances (Schedules II–V) must be prescribed in compliance with DEA regulations, and Oregon Prescription Drug Monitoring Program (PDMP) requirements.
  - Electronic Prescribing (E-Prescribing) is the standard method for prescriptions, including controlled substances. Paper prescriptions are permitted when legally or clinically necessary (e.g., during system downtime, pharmacy limitations, or specific regulatory exceptions). Requirements for electronic prescribing of controlled substances (EPCS) are outlined by the DEA Electronic Prescriptions for Controlled Substances (EPCS) regulations and further clarified in the DEA EPCS Q&A.

- **Clinically Administered Medications**
  - Clinically Administered Medications (CAM) are those given directly to the patient

in the clinic setting (e.g., injections, infusions, implantable medications).
    - CAM differs from prescriptions sent to an outside pharmacy, as the medication is supplied and administered by WHA staff under physician or APC orders.
- **Ordering and Documentation**
    - All CAM must be ordered within Epic using the CAM workflow, ensuring accurate charge capture and clinical documentation.
    - Orders must include medication name, dose, route, frequency, indication, and administration instructions.
    - Administration is documented in Epic by the staff member who gives the medication, with time, route, and dose recorded.
- **Inventory and Storage**
    - Clinically administered medications are stored according to manufacturer guidelines
- **Patient Education**
    - Patients will be counseled regarding purpose, administration, expected effects, and potential side effects prior to administration.
    - Education is documented in the patient's record.
- **Medication Samples**
    - WHA does not maintain samples. If for some reason medication samples are obtained and provided, they must be logged, labeled, and dispensed according to C-20 Sample Medications and Secondary Container Labeling 07.24.23.docx
- **Documentation**
    - All prescribed medications must be documented in the patient's electronic health record (Epic).
    - Documentation must include clinical justification, dosage, frequency, and follow-up plan.
    - Documentation is completed in Epic, including:
- Indication for medication.
- Follow-up plan.

- **Patient Education**
    - Patients will be counseled on medication purpose, dosage, potential side effects, and follow-up.
    - Written instructions or handouts will be provided when appropriate.

- **Compliance and Review**
    - This policy will be reviewed biennially as part of WHA's Clinic Policy Review process.
    - The WHA Medical Director and Clinic Managers are responsible for oversight and ensuring compliance with RHC requirements.
    - CAM practices are subject to internal chart reviews, medication audits, and AAAHC/CMS requirements to ensure proper documentation, safe handling, and accountability.

    - Adverse events or administration errors must be reported according to WHA's Reporting Policy:Safety Event Reporting.docx



Women's Healthcare Associates

Newberg Rural Health Clinic

EMERGENCY OPERATIONS PLAN

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ORGANIZATION INFORMATION

Facility: Women's Healthcare Associates, Newberg Rural Health Clinic

Address: 1003 Providence Dr, Suite 310 and 340

City: Newberg **State:** OR **Zip code:** 97132

Practice Manager: Michele Perez, LPN

Phone Number: 503-855-1564 **E-mail:** mperez@whallc.com

INTRODUCTION TO THE EMERGENCY OPERATIONS PLAN

Purpose: To describe the actions to be taken in an emergency or exercise to ensure that the staff, patients, and visitors are kept safe from harm. The Emergency Operations Plan is developed to align with CMS requirements for Rural Health Clinics. It is informed by a Hazard Vulnerability Assessment specific to Yamhill County and the surrounding region, in which the Newberg Rural Health Clinic (RHC) resides, in order to tailor the plan to hazards recognized as priority due to probability, impact, and mitigation factors.

Policy: Newberg RHC employees and clinicians are responsible for following the procedures described in this policy, and for participating in emergency trainings and exercises. In order to provide for changes in demographics, technology and other emerging issues, this plan will be reviewed and updated biennially.

Background:

a. The clinic is located in Newberg, OR of Yamhill County on the Providence Newberg Medical Center campus (*Tab 1*).
b. The clinic resides across two suites on the 3rd floor of the Medical Center: Suite 310 (2,173 sq ft) and Suite 340 (5,479 sq ft). Each suite contains a reception area, offices, staff break rooms, patient care areas, and restricted storage and utility areas. A floor plan is available at *Tab 2*.
c. The building has appropriate placement and posting of exit signs and fire extinguishers. Emergency exits and fire extinguishers are clearly designated on floor plans.
d. The clinic provides services including obstetrics and gynecology, behavioral health, laboratory, and ultrasound imaging to patients across the lifespan.
e. Yamhill County's population demographics can be reviewed in detail in the *2023 Health Equity Narrative.* Among the County's 108,000 residents, 16.7% identify as Hispanic/Latinx, 8.1% were born outside the United States, 15.2% have a disability, 6.4% are experiencing houselessness, and 5.9% lack health insurance.

EMERGENCY PLAN OVERVIEW

Risk Assessment

a. The clinic conducts a biennial all-hazard vulnerability assessment (HVA) (*Tab 3*). This EOP is developed based on this assessment and is updated as needed to address findings from the biennial HVA, gaps identified during exercises or real events, and changes required by accreditation or regulatory agencies.
b. An electronic copy of the EOP is maintained online on WHA's intranet. A printed copy of the plan is stored in the Practice Manager's office.
c. The prioritized hazards that could affect this facility, as determined by the HVA, are listed in *Appendix A* through *Appendix L*.

Command and Control

a. The clinic maintains an Organizational Chart (*Tab 4*). Accompanying the organizational chart is a Delegation of Authority and Orders of Succession table that will be followed in an emergency. The Delegation of Authority identifies who is authorized to activate the plan and make decisions on behalf of the facility. When an emergency happens, the Incident Response Coordinator, as identified in the organizational chart, will be informed immediately.
b. Depending on the type of emergency, the Incident Response Coordinator will enact the appropriate policies and procedures. Only the Incident Response Coordinator or emergency officials can issue an "all clear" for the clinic, indicating that the facility is ready to assume normal operations.
c. One person, the "Head Counter", will be assigned to list all staff, patients, and visitors that are present in the facility.
d. The Incident Response Coordinator will determine whether to lockdown the facility, shelter in place or evacuate based on the emergency. In the event that the facility must be evacuated, the temporary location for evacuation and facilities for patient transfer are listed in Evacuation Muster Point (*Tab 5*) and Receiving Facilities (*Tab 6*).

Coordination

a. Depending on the emergency, the clinic may need to communicate with outside authorities such as facility security. For immediate threats, like fire or threat of violence, call 911.
b. Local law enforcement and emergency services will coordinate with State, Federal, and other regional agencies as necessary (*Tab 7*).

EMERGENCY MANAGEMENT PROCEDURES

Site Lockdown

a. Situations calling for a lockdown within the clinic may include, but are not limited to: criminal activity or civil disturbance within the facility or the immediate surroundings; threats of violence; infant or child abductions.
b. A site lockdown requires establishing a secure perimeter with controlled entry and exit points. Staff, clients, and visitors will remain in the clinic with all external doors and windows locked. No one is allowed to enter or exit the facility.
c. An overhead code LOCKDOWN page will be initiated, and 911 and facility security must be notified. Staff will then be deployed by the Incident Response Coordinator to key entry/exit point areas to maintain the lockdown process. Additional staff should be deployed to notify patients within the clinic that a lockdown is in process, and they will not be able to exit the building until the lockdown has been resolved.
d. Staff and patients should remain away from windows, and glass doors and blinds will be closed, if safe to do so. It is recommended that all staff, patients, and visitors be secured behind at least two locked doors.
e. The "Head Counter", as designated by the Delegations of Authority (*Tab 4*) will assist in the creation of a list of all employees, patients, and visitors currently on site to be shared with law enforcement. Unless there is an imminent threat, employees, patients, and visitors will be asked to call their emergency contact to let them know where they are and that they are safe.
f. The site will remain in lockdown until the Incident Response Coordinator gives an all-clear. This cancellation may be in conjunction with local law enforcement.

Shelter in Place

a. Shelter in Place means that the staff, patients, and visitors will remain inside the facility's building. Sheltering can be used due to severe storms, violence or terrorism, or hazard material conditions in the area.
b. Windows and doors will be firmly closed and locked where possible. If a storm gets very strong and windows are threatened, staff, patients, and visitors will move to interior rooms and hallways.
c. If sheltering is used in the event of a hazardous chemical incident: windows and doors will be shut, accessible vents will be closed, and accessible air units will be turned off.
g. The "Head Counter", as designated by the Delegations of Authority (*Tab 4*) will assist in the creation of a list of all employees, patients, and visitors currently on site. Unless there is an imminent threat, all persons present on site will be asked to call their emergency contact to let them know where they are and that they are safe.
d. The site will remain sheltered in place until the Incident Response Coordinator gives an all-clear. This cancellation may be in conjunction with local law enforcement.

Evacuation

a. There are a number of hazards that could cause an evacuation: a fire in or near the facility, structural damage from an earthquake, or an evacuation order issued by local authorities.
b. An emergency code will be paged to initiate the evacuation. During an emergency, all occupants must exit from the building using the stairs rather than the elevators. All exits are to be properly marked and free of debris.
    i. If employees are able maintain their own safety while assisting patients and guests, employees are to provide guidance towards evacuation routes. Patient services personnel are responsible for evacuation of themselves and any patients and guests in the reception area. Patient services personnel will also check restrooms near the front office for patients or guests. Clinical services personnel and clinicians are responsible for themselves and any patients and guests in exam and procedure areas.
    ii. To indicate that exam rooms, restrooms, and conference rooms have been checked for patients, magnets stating "CLEAR" will be placed on external door frames. Supervisors, managers, safety committee members or other designees are responsible for placing the magnet on the door frame and shutting the door. This will indicate to other employees and emergency personnel that the room is clear and does not need to be rechecked.

iii. Evacuation of Individuals with Disability or Non-Ambulatory Status: When assisting individuals who are unable to descend the stairs, employees and clinicians should escort individuals to the area of safe refuge (in most cases, the stairwell landing) and notify emergency responders of the location of the individual and what assistance they require. Close any doors between the hazard and the individual's area of safe refuge. Employees and clinicians should not attempt a rescue evacuation unless the individual is in immediate danger and cannot wait for professional assistance.

c. Employees, patients, guests and clinicians must meet at the designated meeting area away from the building (*Tab 5*). The "Head Counter" will account for all employees and clinicians at the designated meeting area.

Suspension of Services

a. In the event that an emergency results in the inability of the clinic to continue providing patient care services, the following plans will be enacted. The decision to suspend services will be made jointly by the Newberg RHC's site leadership and WHA Operations leadership.
b. Clients will be notified that the clinic will not be able to provide services via multiple communication methods (see *Communication Procedures*)
c. The clinic has pre-identified facilities that can deliver required services both internally within the organization and externally in the community. The receiving facilities are listed in *Tab 6*.

Documentation

a. During an emergency, documentation should continue for all patients in the process of treatment.
b. Evaluation should be made on whether to start treatment for patients at the clinic when treatment has not been initiated. The Clinical Response Coordinator (*Tab 4*) is responsible for patient care decisions.
c. All rules pertaining to the protection of and access to patient information (HIPAA) remain in effect during an emergency.
d. Materials for the RHC's downtime documentation procedure are printed and available in the Practice Manager's office.

Volunteers

a. Volunteers will not be used at this facility.

Post-Incident

a. All safety incidents are reported to RLDatix in alignment with WHA's policy *C-28 Safety Event Reporting.*
b. Incidents involving hazardous threats to employees or the facility are reviewed by the Safety Committee.
c. Staff and clinicians involved in crisis response will be offered post-incident support.

COMMUNICATION PROCEDURES

On-Site Emergency Paging

The Newberg RHC utilizes an overhead paging system for communication during emergencies to employees and clinicians in other areas of the office. The following instructions and emergency codes are posted in flip charts and listed on quick-reference cards throughout the clinic:

Overhead Emergency Paging - NB

CALL
Call **503-734-3683** from Teams or cell phone. -OR-
Call *NB Paging* from Teams.

PASSCODE
After two short beeps, enter **7650#**

MESSAGE
After a tone plays overhead, state the **code and location.**

HANG UP
Hang up to end the message.

To end the code, repeat the message followed by "clear"

CODE NAME	DESCRIPTION
Red	Fire
Blue	Cardiac / Respiratory Arrest
Rapid Response	Declining Medical Condition
Orange	Chemical Spill
Grey	Combative Person
Silver	Person with a Weapon
Amber Alert	Missing Infant / Child
External Triage	External Emergency (e.g. Earthquake)
Active Shooter	Gunfire Heard
Lockdown	Secure Site: No Entering or Exiting
Clear	Situation Resolved

When activating a code through the paging system, staff are to state the code type and location. The message should be repeated twice, circumstances permitting. To end the code, start another page and state the code name followed by "all clear"

Off-Site Staff

a. A list of all employees/clinicians and their contact numbers is kept in a secured file cabinet. Access is available to site leadership only.

External to Clinic

a. In the event that phone services are not available, redundant communications may be utilized. Such methods include emergency radios (see *Tab 8* for location and contents of disaster kits) and FlashAlert.
    i. FlashAlert may be accessed by visiting the website at www.flashalert.net. Updates related to the Newberg RHC will be listed under the "Portland-Vanc-Salem" section. These updates can also be found on Portland Metro Area news websites, radio stations, or TV broadcasts.
c. Call "911" for an emergency that threatens the safety or life of staff, clients or visitors. Local law enforcement and emergency services will contact State, Federal, and other regional agencies as necessary (*Tab 7*).

b. WHA Operations leadership will be notified via contact with the Practice Manager. WHA Operations leadership will notify other WHA Executive Leadership Team members.
c. Receiving facilities information can be found under *Tab 6*, including: Contact information for other facilities that can provide patient care services; Contact information for nearby hospitals that can provide emergency services.

Communications with Patients

a. Patients and visitors will be notified about service impacts by WHA Patient Services team members through direct phone calls/messages or bulk MyHealth messages. Additional communication to patients and the community will be available through FlashAlert and/or website postings.

Communications with External Healthcare Providers

a. Only the Incident Response Coordinator or Clinical Response Coordinator, or their designee(s), are authorized to release information on the location or condition of patients. Information may be released to other healthcare providers with consent of the patient and consistent with HIPAA regulations.

Surge Capacity and Resources

a. Based on staffing and patient case loads, this clinic may be available to surge to accept patients from other local outpatient clinics requiring services provided by the Newberg RHC. The WHA Operations team will outline agreements with the sending facilities about the clinic's capacity for surge cases.
b. As requested by local authorities or WHA clinic leadership, the facility will provide excess supplies and/or equipment not needed for their own use.

Requesting Assistance

a. Should the facility require resources to shelter in place, evacuate, or return to service, assistance may be requested from the WHA Operations team or local authorities.

TRAINING

a. All employees and clinicians are trained upon hire and annually in emergency procedures through an Emergency Response online learning module.
b. During Safety Walkthroughs for new employees, Safety Committee members review site-specific evacuation procedures, emergency meeting areas, and emergency response equipment and supplies. Additional training specific to the Newberg RHC EOP occurs on-site with site leadership.
c. Documentation is maintained of annual emergency response module completion, safety walkthroughs, and EOP training sessions. Knowledge of the EOP is demonstrated by participation in the Testing Program.
d. Resources such as flipcharts and quick reference guides are posted at regular intervals throughout the clinic. A copy of the EOP is maintained online on WHA's intranet and a printed copy is stored in the Practice Manager's office.

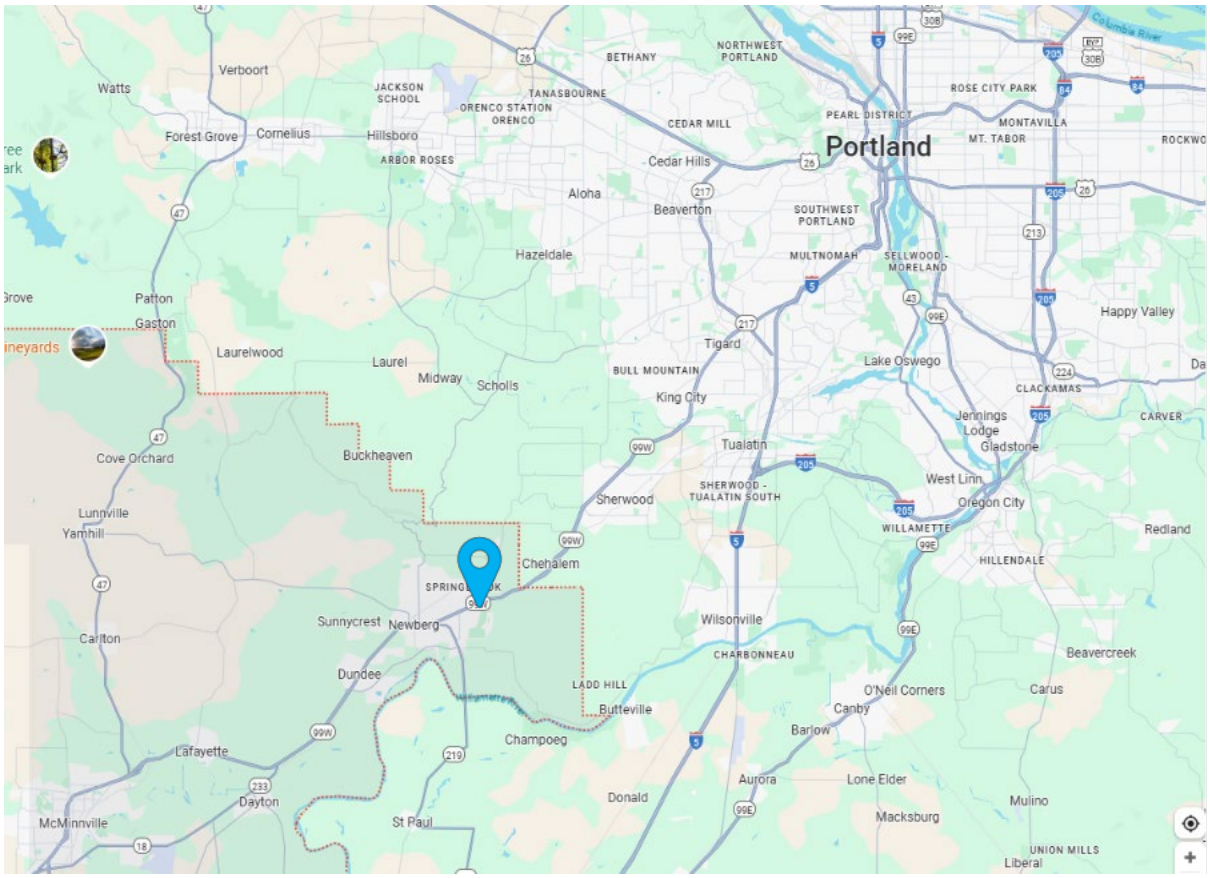
TESTING

a. The clinic will perform a full-scale, facility-based exercise every 2 years. If community-based exercises are available, the clinic will instead participate in an event coordinated with other community organizations.
b. The clinic will conduct an additional exercise every 2 years, opposite the year of the full-scale / functional exercise, that may include a second full-scale or functional exercise; a mock disaster drill; or a tabletop or workshop event.
c. Quarterly emergency response drills are required by organizational policy- although these include both medical / patient care emergencies and facility emergencies and do not fulfill the full-scale exercise requirements for RHCs. Each year, one of the quarterly emergency response drills may be replaced by whichever form of functional exercise is completed that year.
d. If the clinic experiences an actual natural or man-made emergency that requires activation of the EOP, the facility is exempt from engaging in an individual full-scale exercise for 1 year following the onset of the actual event.
e. After full scale exercises, tabletops, or actual events, the clinic should analyze the response, identify areas for improvement, and determine if updates to the EOP are required. A template for review can be found at *Tab 9*.

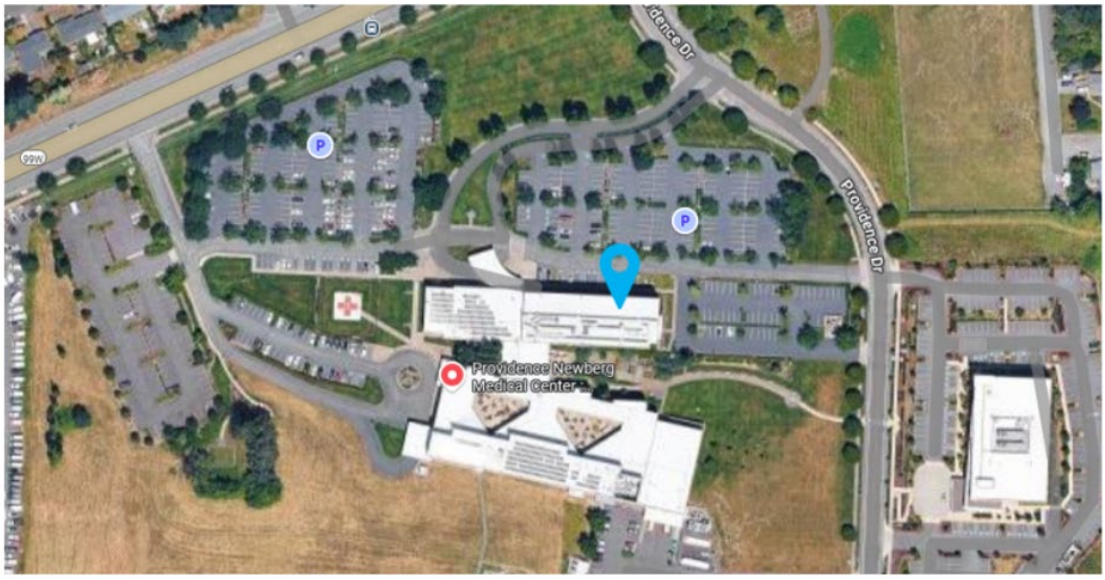
TAB 1

FACILITY LOCATION MAP

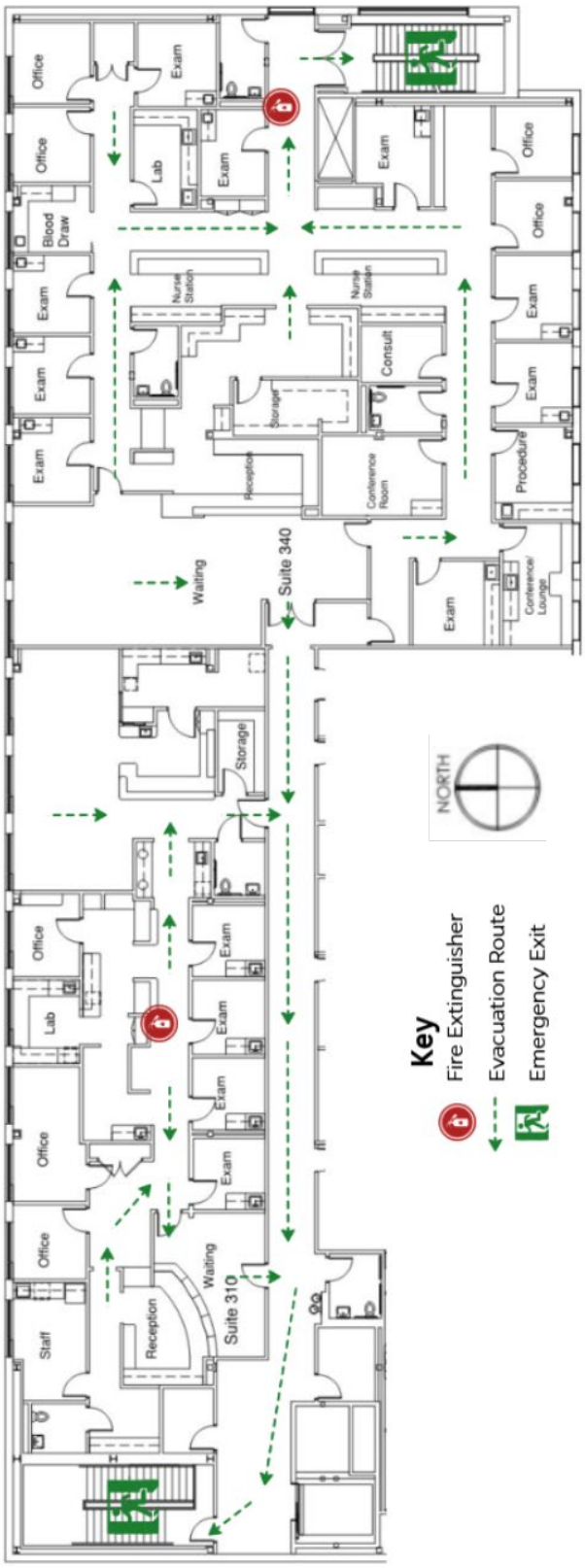
Picture 1: Yamhill county in relation to the Portland Metro Area. Clinic location pinned in blue.



Picture 2: Providence Newberg Medical Center campus with the clinic location pinned in blue.



TAB 2
FACILITY FLOOR PLAN



TAB 3
HAZARD VULNERABILITY ASSESSMENT

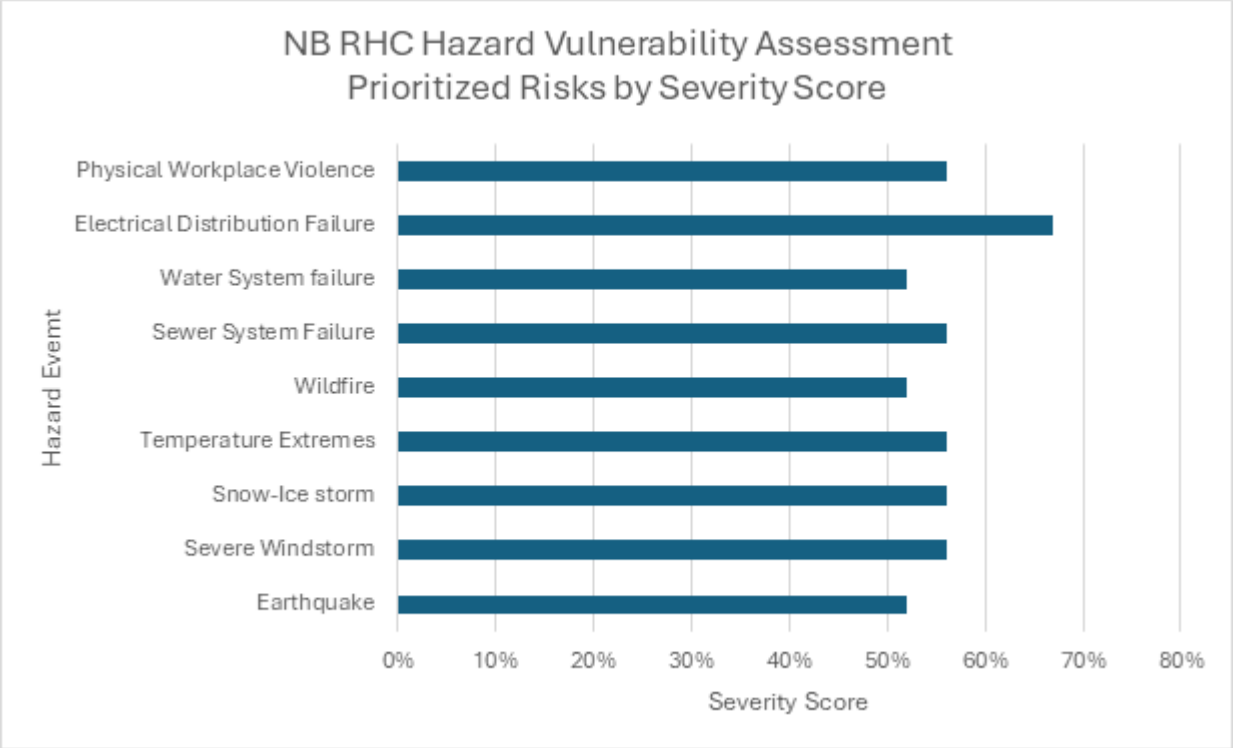
WHA NB RHC Hazard Vulnerability Assessment 2025

HAZARD EVENT	PROBABILITY	SEVERITY = [Probability * (Impact + Preparedness)]						SEVERITY SCORE / RISK
		PATIENT & STAFF IMPACT	HEALTH CARE FACILITY	COMMUNITY	INTERNAL PREPAREDNESS	INTERNAL RESPONSE	EXTERNAL RESPONSE	
	Likelihood of occurrence in 5-year planning window	*Possibility of death, serious injury, or illness to patients or staff.*	*Physical and fiscal losses and damages, impact to daily operations and essential services.*	*Impact on community, partners, local business, and institutions.*	*Policies, training, integrated planning, and exercises.*	*Time, staff, and resouces available.*	*Local emergency management, regional health agencies, community support.*	*Relative risk**
SCORE	*0 = N/A 1 = Low 2 = Moderate 3 = High*	*0 = N/A 1 = Low 2 = Moderate 3 = High*	*0 = N/A 1 = Low 2 = Moderate 3 = High*	*0 = N/A 1 = Low 2 = Moderate 3 = High*	*1 = High 2 = Moderate 3 = Low*	*1 = High 2 = Moderate 3 = Low*	*1 = High 2 = Moderate 3 = Low*	*Scores of 50% or above are flagged.*
Natural Hazards								
Earthquake	2	3	3	3	2	1	2	52%
Pandemic	1	3	3	3	2	2	2	28%
Flood, Local	2	1	2	2	2	2	2	41%
Infectious Disease (SARS, Cdiff, Measles)	3	2	1	1	1	1	2	44%
Severe Windstorm	3	1	2	2	2	1	2	56%
Snow / Ice Storm	3	1	2	2	2	1	2	56%
Temperature Extremes	3	1	1	2	3	1	2	56%
Drought	2	1	1	2	3	2	2	41%
Wildfire	2	2	2	3	3	2	2	52%
Infrastructure Hazards								
Generator Failure	1	1	3	2	2	2	2	22%
Facility Explosion	1	3	3	3	3	3	2	31%
Facility Fire	1	3	3	2	1	1	1	20%
Significant Facility Damage	1	3	3	3	2	2	2	28%
Loss of HVAC	2	1	2	1	2	2	2	37%
Transporatation Failure	2	1	2	2	3	2	2	44%
Roadway System Blocked	2	1	3	3	2	2	2	48%
Sewer System Failure	2	2	3	3	3	2	2	56%
Water System Failure	2	2	3	3	2	2	2	52%
Communications System	2	2	3	3	2	1	2	48%
Fuel Shortage	1	1	2	3	3	2	2	24%
Electric Distribution Failure	3	2	3	3	1	1	2	67%
Technology Hazards								
Data System Failure	1	1	2	0	2	1	3	17%
Electronic Medical Records Failure	2	2	2	0	1	1	3	33%
External Information Systems Failure	2	1	2	0	1	1	3	30%
Cyber Attack	2	1	2	0	1	1	2	26%
Loss of Internet	2	1	2	0	1	1	2	26%
Partner/Vendor Technology Failure	1	1	1	0	1	2	3	15%
Key Equipment Failure	1	1	1	0	2	2	3	17%
Lack of Vendor Availability for Repairs	1	1	1	0	2	2	3	17%
Human & Violence Hazards								
Workplace Violence - Physical	2	3	3	3	2	2	2	56%
Active Shooter	1	3	3	3	2	2	2	28%
Transporation Accident	2	1	1	2	3	3	2	44%
Biological Terrorism	1	3	3	3	2	3	2	30%
Child Abduction	1	1	3	2	1	1	2	19%
Facility Evacuation	3	1	2	1	1	1	2	44%
Bomb Threat	1	2	3	3	2	2	2	26%
Hazmat Hazards								
Radiologic Exposure - Internal	0	0	0	0	3	3	3	0%
Radiologic Exposure - External	2	2	2	0	2	1	2	33%
Hazamat Release - Internal	1	1	2	2	3	3	2	24%
Hazamat Release - External	1	1	2	2	3	3	2	24%
AVERAGE SCORE	1.70	1.60	2.18	1.80	2.03	1.75	2.15	36%

Sources:

- Yamhill County Multi-Jurisdictional Hazard Mitigation Plan, 2020
- Kaiser Hazard Vulnerability Analysis Tool

Prioritized Hazards by Severity Score:



Other Hazards Prioritized for Regulatory / Safety Reasons:

- Hazmat Release
- Bioterrorism Threat
- Facility Fire
- Active Shooter

TAB 4
ORGANIZATIONAL CHART – AUTHORITY AND SUCCESSIONS

Organizational Chart

For online access- See Page 7 of Newberg RHC Policy & Procedure Manual.

For binder access- See next printed page.

Delegations of Authority and Orders of Succession

Task	Primary	Successor 1	Successor 2
Incident Response Coordinator	Practice Manager	Patient Services Supervisor	Patient Care Supervisor
Clinical Response Coordinator (continuity of patient care, interruption of services)	Site Physician Lead	Site APC Lead	Clinician with longest seniority
Head Counter	Patient Care Supervisor	Safety Committee Member #1	Safety Committee Member #2
Contacting EMS / Fire Department / Law Enforcement / Facility Security	Any staff member present, as delegated by Incident Response Coordinator	N/A	N/A
Communications Manager (off-site clinic staff, patients, etc.)	Any staff member present, as delegated by Incident Response Coordinator	N/A	N/A

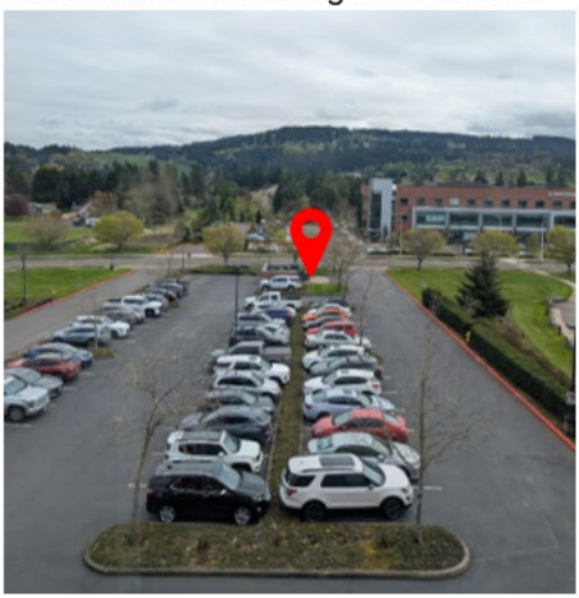
TAB 5
EVACUATION MUSTER POINT

Temporary Evacuation Site / "Muster Point" for clinic staff, patients, and visitors:



East parking lot by the commercial trash bins (dumpsters).

View from an East-facing clinic window



View of the meeting place



TAB 6
RECEIVING FACILITIES

Clinics within the organization who may continue outpatient care for patients:

- **WHA Tualatin** – *12 miles away*
  - Phone: (503) 691-3600
  - Address: 19250 SW 65th Ave, Suite 300, Tualatin OR 97062
- **WHA Peterkort** – *20 miles away*
  - Phone: (503) 734-3777
  - Address: 9701 SW Barnes Rd, Suite 200, Portland OR 97225
- **WHA Oregon City** – *22 miles away*
  - Phone: (503) 905-4343
  - Address: 1508 Division St, Suite 205, Oregon City, OR 97045

Community Hospitals who may offer emergent care:

- **Providence Newberg Medical Center** – *0 miles away*
  - Phone:
    - (503) 537-1555 (General campus contact)
    - (503) 537-1758 (L&D unit)
  - Address: 1001 Providence Dr, Newberg OR 97132
- **Legacy Meridian Park Medical Center** – *12 miles away*
  - Phone:
    - (503) 692-1212 (General campus contact)
    - (503) 692-7500 (L&D unit)
  - Address: 19300 SW 65th Ave, Tualatin OR 97062
- **Willamette Valley Medical Center** – *15 miles away*
  - Phone:
    - (503) 472-6131 (General campus contact)
    - (503) 435-6400 (L&D unit)
  - Address: 2700 SE Stratus Ave, McMinnville OR 97128

TAB 7
CONTACT INFORMATION

Staff and Clinicians

Names and cell phone numbers for all Newberg RHC staff are stored in a secured location within the clinic, which site leadership retains access to. Organizationally, WHA Operations leadership and Human Resources may pull contact information from personnel files when required.

Facility Contacts

Providence Security: (503) 537-1899

Providence Facilities: Practice Manager to directly contact Facility Managers with CBRE

Regional Contacts

Newberg-Dundee Police, Non-Emergency Line: (503) 538-8321

Yamhill County Mobile Crisis Team: (844) 842-8200

Yamhill County Sheriff, Non-Emergency Line: (503) 434-6500

Yamhill County Emergency Management: (503) 434-7340

Oregon Department of Emergency Management: (503) 378-2911

FEMA Region 10, General Line: (425) 487-4600

TAB 8
DISASTER KIT CONTENTS AND LOCATION

There are two disaster kits stored at the Newberg RHC, one in each suite:

- Suite 340: Storage closet.
- Suite 310: Top shelf of the cabinet across from the lab.

Contents include:

- General supplies-
  - Garbage bags
  - Roll of plastic sheeting
  - Heavy duty gloves
  - Emergency radio (crank model)
  - Solar battery phone charger
  - USB-C charging cable
  - Duct tape
  - Flashlight
  - Whistle
  - Emergency blankets
  - Light sticks
  - Extra batteries
  - Multi-function tool
  - Manual can opener
  - Dust masks
  - Waterproof matches
  - Bleach
  - Dropper (for bleach)
  - Emergency food ration
- Medical supplies-
  - General first aid kit
  - Towelettes
  - 250 ml saline (sterile bottle)
  - Baby wipes
  - Paper tape
  - Alcohol prep pads
- Resource binder-
  - Local emergency contacts
  - Shelter in place guide
  - Water disinfection guide
  - Regional evacuation map

TAB 9
AFTER-ACTION REVIEW and IMPROVEMENT PLAN

Follow the template provided by HHS at:

https://asprtracie.hhs.gov/technical-resources/resource/185/health-care-provider-after-action-report-improvement-plan-aar-ip

APPENDIX A
FACILITY FIRE

Preparation:

1. Familiarize yourself with the location of fire extinguishers and fire alarm pull boxes in your area. These will be identified during your safety walkthrough.
2. Participate in fire drills.
3. Extension cords should be used only on a limited, short-term basis. They should be visibly inspected at the beginning of each job.
4. Turn off electrical appliances when not in use, when appropriate.
5. Do not bring hazardous materials into the building without prior approval, including chemicals or appliances not approved for use at WHA.
6. Properly dispose of flammable materials.
7. Keep clutter to a minimum, and maintain clear walkways at all times.

Response:

In the event of a fire emergency, employees and clinicians are to immediately inform all other persons in the office by announcing the need to exit the building. Emergency fire alarm pull stations should be activated. Once the alarm is given, 911 should be called while evacuating. If the building alarm is not already ringing, then the first employee or clinician to pass a pull station is to activate the alarm.

Even if clinicians or employees believe an alarm to be a false alarm or a drill, all clinicians, employees, patients, and guests must exit the building, and are to wait for clearance from the Incident Response Coordinator or emergency personnel before re-entering. See the *Evacuation Procedure* for additional details.

Only personnel who have been trained to use the fire extinguishers are authorized to use them in an emergency, except to prevent an imminent threat to safety. Safety Committee members are provided fire extinguisher training annually.

Types of warning equipment used at the clinic include smoke detectors, auditory alarms, and verbal alarms. The Newberg RHC is equipped with fire extinguishers, sprinkler systems, and first aid equipment. Automatic sprinkler systems, fire detection systems, alarm systems, and fire doors shall be maintained in continuous proper operating condition.

In summary, staff and clinicians are asked to follow the RACEE acronym when responding to a facility fire emergency:

- **Rescue** – Remove patients and visitors from immediate fire scene
- **Alarm** – Pull the fire alarm and call 911. Give the following information: your name, building address, phone number, location of fire, status of evacuation.
- **Contain** – Close all doors and windows
- **Extinguish** – If you are extinguisher trained and if the fire is small, contained, and not blocking your exit, use the **PASS** Procedure:
    - Pull the pin
    - Aim the nozzle at the base of the fire
    - Squeeze the handle
    - Sweep from side to side
- **Evacuate** – Evacuate all patients and guests to emergency meeting place using stairs. Check for smoke / fire before opening doors. Mark room doors with the CLEAR magnets when cleared of all persons.

APPENDIX B
WORKPLACE VIOLENCE

WHA and the Newberg RHC strive to provide a safe workplace for all employees, clinicians, and patients. In doing so, the Newberg RHC does not tolerate any type of workplace violence committed by or against any employee or clinician. The following list, while not all inclusive, provides examples of conduct that is prohibited under this policy:

- Voluntary and intentional swearing and/or making derogatory, discriminatory, harassing comments towards any WHA employee, clinician, visitor or other patient (including sexual harassment);
- Refusal to comply with WHA's policy regarding weapons in facilities;
- Voluntary and intentional threatening or aggressive behavior, including behavior that is directed towards property, with the intention or impact of intimidating another person while at a WHA facility;
- Voluntary and intentional sustained yelling or screaming in a WHA office or while on the phone with a WHA employee or clinician;
- Verbal or written threat of a direct act of violence against any WHA employee, clinician, visitor or other patient; and
- Voluntary and intentional unwanted/uninvited physical touch of others at any WHA facility.

Any employee or clinician determined to have committed any such conduct will be subject to corrective action, up to and including termination of employment. If such behaviors are exhibited by a patient, any employee or clinician may initiate a request to consider of discontinuation of care.

Prevention

Emergency response drills are conducted at least quarterly and full-scale exercises are conducted annually. These may incorporate workplace violence scenarios.

Patients are notified of conduct expectations towards staff, clinicians, and visitors at the initiation of their care with WHA through acknowledgement of the Patient Rights & Responsibilities document.

Response and Mitigation

An emergency code may be activated in applicable workplace violence events. See the *On-Site Emergency Paging Procedure* for additional details. Relevant codes may include code GREY, code SILVER, and/or code ACTIVE SHOOTER.

Safety measures to prioritize during a workplace violence event include the following:

- Call for assistance
- Maintain safe distance from the individual
- Secure tools that could cause harm
- Redirect the individual to an area away from patients
- Use the buddy system. If you must remain alone with the individual, maintain a clear exit route.
- Utilize de-escalation techniques such as- Speaking calmly, clearly, and slowly; Demonstrating empathy; Validating their concerns; and Offering choices.

The Newberg RHC is encouraged to use internal and external resources in the management of a workplace violence event, including but not limited to Site Leadership, Behavioral Health Clinicians, campus security, law enforcement, and mobile crisis teams. In Yamhill County, mobile crisis team services are limited but can be reached directly at 844-842-8200. The Providence Newberg Medical Center security team can be reached at 503-537-1899. See *Tab 7* for additional contacts.

Additional response measures may include the convening of an incident review group with members from Site Leadership, Behavioral Health Clinicians, Operations, Risk & Compliance, Human Resources, and Employee Health departments.

APPENDIX C
ACTIVE SHOOTER

Preparation:

1. Be aware of your surroundings.
2. Know where exits are located throughout your site.
3. Know which internal doors are able to be locked. At the Newberg RHC, locking doors include the below. Locking doors are indicated with the presence of a green magnet.
    a. Doors between the lobby and the back office
    b. Bathroom doors
    c. Some office doors
    d. Suite 340 back staircase (entry locked only, exit remains open at all times)

Response:

When there is an Active Shooter in your vicinity, there are three options: Run, Hide or Fight. These are in order of priority and preference.

- Evacuate / **Run**
    - Have an escape route in mind
    - Evacuate regardless of whether others agree to follow
    - Leave your belongings behind
    - Help others to escape, if possible
    - Follow instructions of law enforcement
    - Do not attempt to move wounded people
    - Call 911 and activate Code ACTIVE SHOOTER when you are safe
- **Hide**
    - Be out of the active shooter's view
    - Pick a spot where you will be protected if shots are fired, such as a closed and locked office door
    - Blockade a closed door with furniture
    - Lay on the ground
    - Silence your cell phone and text 911, if safe to do so
    - Remain quiet
    - Turn off lights
- **Fight** / Take Action
    - Act as aggressively as possible against the shooter, and commit to your actions
    - Throw items
    - Improvise weapons (fire extinguishers, chairs, etc.)
    - Yell

Once the Police have arrived, keep hands visible and raise over your head. Provide information about location of shooter, wounded and description of shooter, if known.

APPENDIX D
ABDUCTED CHILD or INFANT

Response:

- Obtain the child's name from parent(s)/guardian(s), but do not release it via overhead paging.
- Immediately call 911 and facility security.
- Page Code AMBER ALERT or Code LOCKDOWN
- Designate someone to stay with the parent(s)/guardian(s) in private area.
- In addition to the steps described in the *Site Lockdown Procedure* described previously:
    - Observe all persons leaving and record vehicle license plates.
    - Ask individuals with children not to leave until law enforcement arrives.
- Search measures:
    - Activate a search of the interior and exterior of the suite.
    - Search people who are carrying large bags, or ask them to remain in place until law enforcement arrives.
    - Question the parent(s) of the missing child as to other possible locations of the child in the facility.
    - Staff should not attempt to restrain an uncooperative subject. Instead, report a description and location of the subject to law enforcement

APPENDIX E
UTILITY LOSS – WATER or SEWAGE

If water supply or sewage systems are disrupted for any reason, the following steps will be taken. Additional guidance can be found in the *Infection Control Risk Assessment for Utility Failures* document, which provides instructions for maintaining the integrity of infection prevention practices for the facility, equipment, and supplies during a water supply or sewage management utility loss.

Water Supply Outage

- All attempts will be made to determine the cause for water disruption and the probable length of shutdown.
- If potable water is needed immediately, use bottled water. In a shelter-in-place scenario of longer duration, follow the drinking water disinfection guide in the disaster kits.
- Access to water is required for patient care, employee safety, and cleaning, disinfection, and sterilization measures. In the event of an extended water supply constraint, Newberg RHC's site leadership will coordinate with WHA Operations leadership to determine the ability to continue operations on site.

Sewage Management Interruption

- All attempts will be made to determine the cause for the sewage utility loss and the probable length of shutdown.
- Identify and label working and non-available toilets.
- If a wastewater backup occurs, small and/or contained spills may be cleaned by clinic staff utilizing the appropriate PPE and disposal measures. Providence Facilities should be notified to respond to large spills.
- Restrict access to areas impacted by wastewater backups.
- Access to working toilets is required for patient care and employee safety. In the event of an extended sewage utility loss, Newberg RHC's site leadership will coordinate with WHA Operations leadership to determine the ability to continue operations on site.

APPENDIX F
ELECTRICAL POWER OUTAGE

In the event of a power outage, the following steps will be taken.

- Determine the amount of time that power is expected to be out, and whether the power company will take immediate steps to restore power to healthcare facilities such as the Providence Newberg Medical Center.
- Verify that back-up systems are working and supplying power to necessary equipment.
- Monitor refrigerator temperatures. If the temperature of the fridge is trending upward, or if the power outage is expected to last 4 hours or more, make plans for transporting vaccines and/or medications to alternative cold storage.
- Access the disaster kit for additional sources of light, if needed.
- Electrical power is required for patient care and employee safety. In the event of a power outage, Newberg RHC's site leadership will coordinate with WHA Operations leadership to determine the ability to continue operations on site.
- Reference the *Infection Control Risk Assessment for Utility Failures* for additional guidance specific to maintaining infection prevention practices for the facility, equipment, and supplies during a power outage.

APPENDIX G
EXTREME TEMPERATURES

In most cases of extreme temperatures, the clinic's and medical center's HVAC system is designed to maintain comfortable indoor temperatures within the facility. In the event of a failure of the HVAC system, temperature excursions outside of the comfortable zone may occur. Suggested measures for these moderate excursions are listed below. Extreme excursions will result in Newberg RHC's site leadership coordinating with WHA Operations leadership to determine the ability to continue operations on site.

Precautionary Procedures for Extreme Temperatures:

Heat

- Draw all shades, blinds and curtains in rooms exposed to direct sunlight.
- Supply ample potable fluids for both patients and staff members.
- Utilize fans or portable AC units to keep air circulating, while observing electrical device safety guidance.
- Concentrate patient care in areas of the clinic that exhibit the most effective temperature management.

Cold

- Space heaters may be used with the approval of Providence Facilities.
- Precautions for space heaters:
    - The heating element should be protected from accidental touch to prevent burns.
    - The space heater should be 3 feet away from any flammable materials.
    - Plug the heater directly into a wall outlet, do not use an extension cord. Ensure the cord does not pose a tripping hazard.
    - Do not leave the heater unattended while on.
- Safety features required for space heaters:
    - Portable, with handle to move the heater without touching a hot element
    - Adjustable temperature
    - Automatic shut off for overheating
    - Automatic shut off for tipping over

APPENDIX H
SEVERE WEATHER & WINTER STORMS

Inclement Weather and Severe Conditions Prior to Clinic Opening

It is the objective of WHA's Newberg RHC to be accessible to our patients whenever possible and appropriate. There may be instances where the weather limits operations and patient care.

The Practice Manager, in coordination with WHA Operations leadership, is responsible for determining the necessity of closing the site due to weather conditions. Site closure shall only occur in extreme conditions. The decision shall be communicated via the methods described under the *Communication Procedures*.

Employees are expected to make every reasonable effort to come to work. Newberg's RHC recognizes individuals need to make decisions about coming to or leaving work
based on their individual circumstances.

Severe Conditions During Clinic Operating Hours

- Account for all patients and staff on duty. Make sure everyone is inside.
- Make sure that windows are locked and secured.
- Keep all patients, staff and visitors away from external doors and windows.
- Notify patients if services will be canceled.

APPENDIX I
WILDFIRE

Site leadership will monitor regional emergency communications for updates when wildfires are considered a threat. Yamhill County's Emergency Alert System will provide notifications when evacuation levels are advanced to a higher rating. Evacuation levels utilized by the State of Oregon and the respective preparedness measures are below.

Level 1 – Be Ready

- The Newberg RHC will maintain all emergency systems to be ready for activation at any time, as described throughout this EOP.

Level 2 – Be Set

- Newberg RHC's site leadership will coordinate with WHA Operations leadership to determine the continuity of operations on site. If services are to be interrupted, the *Communication Procedures* will be followed to notify patients and staff not on site.
- If patient services continue-
    - Means for evacuating from the clinic will be maintained as described in *Evacuation Procedures*. In order to reduce potential supply-loss related to evacuation, arrangements will be made for the transfer of excess temperature-controlled supplies.
    - Considerations for air quality management may include but are not limited to: communication with Providence Facilities about HVAC measures; installation of standalone air filtration systems; closure of external windows and doors; provision of respiratory PPE to patients and staff.
    - Staff may receive orders for evacuating from their homes before the clinic does. If staffing shortages occur, the Receiving Facilities listed on *Tab 6* may assist with reducing the patient care load, and other WHA clinics may offer staffing support.

Level 3 – Go Now

- Immediately implement the *Evacuation Procedures* detailed previously.
- Initiate all necessary *Communication Procedures* detailed previously.

APPENDIX J
EARTHQUAKE

Preparation:

1. Secure shelves, bookcases, file cabinets, and other top-heavy furniture.
2. Store breakable and heavy objects on lower shelves.
3. Contact Providence Facilities to shut off power or gas supply.
4. Block all wheels on large items such as ultrasound equipment.
5. Ensure that there is adequate room under desks or workstations to take cover.

Response:

Drop

- Drop down to your hands and knees
- Do not run outside. If you are outside, stay outside.
- Watch for falling lights, ceiling tiles, bookcases, filing cabinets, etc.
- Stay away from external windows.

Cover

- Take cover under a desk, table or counter. If there isn't a table near you, crouch against an interior wall.
- Cover your head and neck with your arms.
- DO NOT get into a doorway

Hold-On

- Hold on to your shelter (and/or to your head and neck) until the shaking stops.
- Be prepared for strong aftershocks. Move items that may fall to the floor.
- Decisions regarding evacuation should be made upon an assessment of present risks. Do not evacuate unless told to do so by the Incident Response Coordinator or another official. Open doors slowly during evacuation.
- Use the radio from the disaster kit to obtain additional information.

Additional Considerations

Patient Care

- Advise patients and visitors to drop, cover, and hold-on.
- If safe to do so, or if there is an alert sent prior to an earthquake, consider the following measures for incapacitated patients:
    - Use pillows, blankets, or other linens to provide a barrier to falling debris. Prioritize coverage of the patient's head and surgery site (as applicable).
    - Lower the patient's bed to the lowest position.
    - Move the patient close to an internal wall, and away from cabinets, shelving, or equipment that may fall
    - Do not attempt to move the patient during the shaking unless there is imminent danger.

APPENDIX K
HAZMAT INCIDENT – INTERNAL SPILL

Chemical Spill Response Steps

- Initiate an emergency page to announce the code ORANGE
- Stop foot traffic near the area
- Post a spill / slip hazard sign
- Retrieve the Safety Data Sheet (SDS)

  Find the SDSs on SharePoint's Safety Page

  -OR-

  Contact BL to access physical copies in case of outage
- Go to **Section 6 – Accidental Release Measures** of the SDS for spill clean-up instructions
  - If evacuation of the area is required, follow the *Evacuation Procedure* steps and contact emergency services.
- Gather clean-up supplies and don appropriate PPE
  - Do not use "Red Z" from Bloodborne Pathogen Spill Kits on chemical spills.
  - Supplies like absorbent mats or chemical spill kits may be used to clean up chemical spills.
- Go to **Section 13 – Disposal Considerations** of the SDS for material disposal instructions

APPENDIX L
BIOTERRORISM THREAT

Recognizing a Bioterrorism-Related Event
As an outpatient healthcare clinic, the Newberg RHC will have limited scope in recognizing epidemiologic features of bioterrorism events. Employees and clinicians must still remain alert to the potential of individuals exposed to agents external to the organization, or to multiple patients and/or employees exposed internally within a WHA site. A bioterrorism-related event may occur as a covert event, in which persons are unknowingly exposed and an outbreak is suspected only upon recognition of unusual disease clusters or symptoms. Bioterrorism may also occur as announced events, in which persons are warned that an exposure has occurred.

Covert events must be reported immediately and may present with a rapid increase with similar symptoms seeking medical treatment especially if they live or work in a common geographical area.

Announced event response would be coordinated by the FBI and local law enforcement agencies. Information relevant to the operations of the Newberg RHC would be communicated by public health personnel of local health departments, i.e. Yamhill County Public Health. Such information may include instructions for implementing enhanced surveillance for suspected disease among patients. Syndrome-based criteria is expected to be described by local health departments during an announced bioterrorism event.

Reporting Requirements and Contact Information
Any suspected case of bioterrorism shall be treated as a public health emergency and reported to the site leadership, WHA administration, and infection control. Internal notifications shall be made immediately through the overhead paging system. External contacts may include Yamhill County Public Health (Emergency line: 503-474-3194) and local law enforcement. Clear, consistent, understandable information shall be provided to patients, visitors, and the general public by WHA's Patient Services and Marketing departments.

Exposure Management
All patients, employees, and clinicians exposed to a biological or chemical agent of terrorism will immediately receive EMS transportation to the nearest hospital. If the source of the exposure is suspected to be within a WHA site, an emergency evacuation will be initiated according to the steps described previously. All further measures will be taken under the instruction of the local public health authority and local law enforcement.

Infection Control Practices
Agents of bioterrorism are generally not transmitted from person to person; re-aerosolization of these agents is unlikely. All patients, including symptomatic patients with suspected or confirmed bioterrorism-related illnesses, should be managed utilizing Standard Precautions (see *Policy S-4 Exposure Control Plan*). For certain diseases or syndromes (e.g., smallpox and pneumonic plague), additional precautions may be needed to reduce the likelihood for transmission. Decisions regarding the need for specific isolation precautions and infection control practices will be made in consultation with the local public health authority.

Decontamination of the Environment
Decisions regarding the need and protocol for decontamination will be made in consultation with the local public health authority.

Inter-Agency Cooperation
WHA will cooperate with all FBI, CDC, local public health authority, and local law enforcement requests for scene securement, evidence collection, witness statements, and public communication.

---END OF EMERGENCY OPERATIONS PLAN---

Additional Policy Adoption Statement

In addition to the developed Rural Health Policies, the Newberg Rural Health Clinic (RHC) will also adopt the following Women's Healthcare Associates (WHA) organizational policies to ensure alignment with best practices and continuity of care across all locations:

The following WHA policies will be adopted by the Newberg RHC:

Medical Emergency Response
Sample Medications & Secondary Container Labeling
Environmental Surface Cleaning for Exam, procedure and Lab
Hand Hygiene
Medication Variance and Error Reporting
Patient Incident Reporting
Vaccine Management
Safe Medication Practices, Administration, and monitoring requirements
Maintenance and Electrical Safety Testing of Ultrasound Equipment
Code of Conduct
Employee Records
Equal Employment Opportunity- Hiring Process
Medical Assisting Certification
Confidentiality
Laptop and Portable Device Security
Access and Authentication
Information Systems Security
Payment Card Industry Security
Corrective Action & Training Responsibility for HIPAA
Patient Complaint
Records Retention
Release of Protected Health Information
Language Access and Nondiscrimination
Safety
Respiratory Protection Program
Exposure Control Plan
Tuberculosis Control Plan
Hazard Communication Plan
Decontamination and Sterilization

These WHA policies will be upheld alongside the Rural Health Policies and are considered integral to the clinical and operational standards of the Newberg RHC.

Rural Health Policies Signature Page
Approval of Policy Review by Rural Health Committee Members

We acknowledge that we have reviewed the current Rural Health Policies. By signing below, we affirm that these policies have been reviewed and approved in accordance with the standards and procedures set forth by the Rural Health Committee.

Title	Signature	Date
Chief Medical Officer	_____	_____
Physician Site Lead	_____	_____
Advanced Practice Clinician (APC)	_____	_____
Nurse Manager	_____	_____
Practice Manager	_____	_____